

إسم المريض رباعياً : راندا رمزي رزق حنا
المحافظة : الجيزة الحي / المدينة : الهرم
رقم البطاقة : 26306020300086 رقم التليفون : 01224995066
العمر(تاريخ الميلاد) : 02-06-1963 المهنة : ربه منزل
العنوان : رشدي
تاريخ تسجيل الإستمارة : 26-09-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)

خطه العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضى الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157]

التعليق على التشخيص : -----

قسم العلاج : الاورام

الاسم: د/محمد كيلاني الوظيفة: _____
 الاسم: د/ندى حسن الوظيفة: _____
 الاسم: د/نرمين رمضان الوظيفة: _____
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: راندا رمزي رزق حنا National ID: 26306020300086	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



الى من يهمه الامر / مستشفيات جوسناف رئيس

تحية طيبة وبعد....

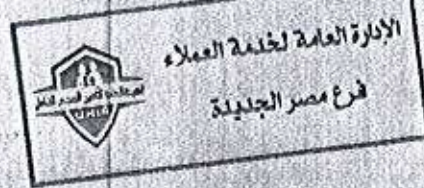
بالإشارة الى قانون الهيئة العامة للتأمين الصحي الشامل رقم 2 لسنة 2018 ، والذي يتضمن بوجوب خضوع المقيمين بمحافظات التطبيق اجباريا بالمنظومة وفقا لمحل الاقامة ببطاقة الرقم القومي داخل احدي محافظات التطبيق، وبالإشارة الى بدء التشغيل الفعلي (2019\7) لمنظومة التأمين الصحي الشامل بمحافظة (بورسعيد)

برجاء التفضل بالعلم بأنه قد تم الاستعلام عن بيانات المواطنة / راندا رمزي رزق حنا الله تحمل بطاقة الرقم (26306020300086) المرفقة والتي قد تم اصداؤها في محافظة الجيزة بناء علي ما سبق في ان المستفيد المذكور من غير الخاضعين لمنظومة التأمين الصحي الشامل

وتفضلوا بقبول فائق التقدير والاحترام...



تاريخ: 2026/03/15
ع من جمهورية مصر العربية لشئون المستفيدين



جمهورية مصر العربية
بطاقة تحقيق الشخصية



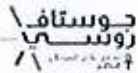
رائدا
رمزى رزق حنا الله
٢٠ سن فاطمة رشدى
الهرم - الجيزة
٢٦ ٠٣ ٠٢ ٠٩ ٦٣ ٢



IW6195913

bone scan -
معملية على الصدر

Name:	رانداء رمزي رزق	
MR. :	5125	
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 62-year-old individual with de novo metastatic breast cancer, estrogen receptor (ER) positive, progesterone receptor (PR) positive, and HER2 negative, presenting initially in the emergency department with bone metastases. Palliative radiotherapy was delivered to the lumbar spine (L1-L5). Since April 2025 the patient has been receiving letrozole (Femara) and zoledronic acid (Zometa) every three months, with the next scheduled Zometa dose on 22 April 2026. The most recent assessment, performed two months prior without direct patient contact, was for continuation of letrozole therapy. The patient has a history of left breast invasive ductal carcinoma (IDC) grade 3, with ER 8/8, PR 6/8, HER2-negative disease, metastatic to bone. In 2025 the patient sustained a left femur fracture requiring internal fixation and received palliative radiotherapy to the upper left femur (L3-L4). Since April 2025 the patient has been maintained on letrozole and zoledronic acid. The last oncologic evaluation in November 2025 noted stable disease. More recently, new left breast skin nodules and fungating masses have developed, prompting multidisciplinary discussion and a plan for continued zoledronic acid and letrozole therapy. Multidisciplinary team (MDT) review considered a regimen of palbociclib (Ibrance) plus fulvestrant; however, the required QARA (quality assurance review) is pending. Imaging on 14 May 2026 demonstrated an ill-defined soft-tissue mass inseparable from the skin in the left breast measuring 6.9 × 5 cm. A bone scan performed on 18 May 2026, compared with the study from 12 2025, showed stable disease. Pain management consultation was provided. On 20 April 2026 the MDT discussed initiation of Faslodex (fulvestrant) and Ibrance. Physical examination revealed an ulcerative left-breast lesion with skin nodules measuring approximately 4 × 5 cm. The patient received Faslodex 500 mg and Ibrance 125 mg on day 1 of cycle 1, along with zoledronic acid and vitamin D supplementation as part of the new treatment plan. The patient is on day 1 of cycle 1 of Faslodex and Ibrance.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
	dina makram	الطبيب المعالج :
يعتمد ,		

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

04/20/2026

MDT FORM

Name:	Randa Ramzy Rizk	MRN:	5125
Age:	63 Y	ID:	26306020300086
Menopausal Sta	postm	Phone:	01002781454
NRS SCORE:	ps3	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

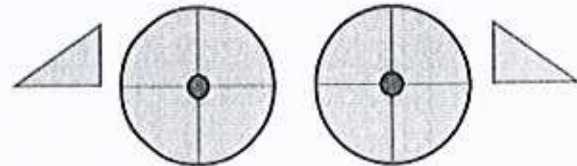
BX: R L
ER: PR: HER2: KI67:

Sonomammography:Mets Workup:History:

postm left breast cancer Idc 3 er8/8 pr6/8 her2-ve met to bone presented in 2025 by left femur fracture and internal fixation rec pall rth to upper left femur I3 I4 st femara and zometa from 4/2025 last evaluation 11/2025 sd recently newly developed left breast skin nodules and fungating masses for mdt for new cts bs

Recommendations:MDT Outcome & Plan

FASLODEX +IBRANCE
NEW CTS AND Bone Scan



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نورهين مصطفى					

Name : Randa Ramzy Rizk Hana Allah

Date : 18-05-2026 06:21

Branch : Dokki

Medical No. : 377276

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاقد مستشفيات

Tc-99m MDP Whole-Body Skeletal Scintigraphy

- Indication: 62-year-old female patient. Follow-up of known metastatic breast cancer on hormonal therapy; evaluation of skeletal disease burden and assessment of response to treatment.
- Comparison: Prior scintigraphy reports dated 01/12/2025 and 15/03/2025.

Technique

Following the intravenous injection of tc-99m MDP, anterior and posterior whole-body planar scans were acquired approximately 2 hours post-injection.

Findings

- **Spine:** Persistent, multifocal areas of increased tracer uptake are noted throughout the upper thoracic, and lumbar vertebrae, showing no significant change in spatial distribution or tracer avidity.
- **Thorax:** Stable focal hyperfixation is noted in multiple bilateral ribs (predominantly involving the posterior and lateral segments of the lower left ribs), matching the patterns seen in prior exams. Mild focal uptake is still seen in the lower sternum.
- **Shoulder Girdle:** Stationary focal uptake is observed in both scapulae and both shoulder joints, partly related to known secondary osseous involvement with superimposed degenerative arthropathy.
- **Pelvis & Lower Limbs:** Intense, localized tracer accumulation is noted in the left proximal femur and intertrochanteric region, demonstrating a stable appearance consistent with chronic reactive bone remodeling/tracking around the reported internal fixation hardware. A stable, minor focus is noted in the right distal tibia/ankle. No new focal lesions are identified in the remaining long bones.

Impression

- **Stable Skeletal Metastatic Disease:** Multiple osteoblastic skeletal metastases involving the spine, sternum, bilateral ribs, scapulae, and left femur, demonstrating a more or less stationary course with no significant interval changes in tracer avidity or spatial distribution compared to prior studies. No new distinct lesions could be depicted.
- **Stable Post-Surgical Changes:** Persistent, expected reactive hyperfixation in the left proximal femur, consistent with chronic remodeling secondary to internal fixation hardware.

Much oblige
 Dr. Alaa Nagi, MD
 Nuclear medicine

Patient Name : لاسناء / رائدا رزق غنا الله
Patient No. : 26306020300086
Path No. : 2773/24
Sex : Female
Age : 61 Y
Referring Doc. :

Request No. : 13008348
Request Date : 05/08/2024
Verification Date : 18/08/2024
Financial Category : نفقة الدولة - Out Patient
Department :
IHC No. :
Registered by : marwa mohamed you

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 05/08/2024, 09:45 am.

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5). & Ki67 clone (30-9).

Results:

IPT (Positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8 /8).
PR: Mild nuclear reaction in 70% of tumor cells (Allred score 6 /8).
Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+).
Ki67: 22% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade III.

ER: Positive (score 8 /8).
PR: Positive (score 6 /8).
Her2: (score 1+).
Ki67: Positive in 22% of tumor cells.

Scoring system: "ER, PR"

Verified by :
DR.Menna Nabil

Printing Date : 20/10/2024 11:47:36

Page 1 of 2

Printed By : marwa mohamed youss



Patient Name: Randa Ramzi Rezk Hanallah	Date : 15/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 61 Year(s)
Medical ID: 462/2025	Pat. ID: 97700

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Left breast pathologically proven invasive duct carcinoma grade II.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- ACR:A.
- Left breast LIQ infiltrative irregular dense lesion retracting the nipple with diffuse skin thickening.
- Complementary US revealing an irregular hypoechoic mass lesion measuring 6.2 X 3.5 cm occupying most of LIQ extending from 7 to 9 o'clock invading the overlying skin ,about 1.4 from the nipple.
- Right UOQ focal asymmetry complementary US revealing normal glandular tissue.
- No right breast suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- No right breast solid or cystic mass lesions.
- Normal skin thickness and nipple-areola complex of right breast.
- Few prominent left axillary level I lymph node showing thickened cortex ,largest measures 1.3X0.9 cm.
- No sizable left infra or supraclavicular lymph nodes.

Opinion:

- Left breast LIQ pathologically proven invasive duct carcinoma showing infiltrative malignant mass as described (BIRADS 6).
- Left axillary level I indeterminate lymph node for FNAC.

Yours truly

Signature Dr. Youmna tarek

15/02/2025 12:20 PM

Nuclear Medicine Unit

Patient Name: Randa Ramzi Rezk

Date: 15/3/2025

Referring physician: Dr. T. El-M **99mTc-MDP BONE SCAN**

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- Multiple marrow based active metastatic osseous lesions are seen opposite the sternum , bilateral ribs , both scapulae , multiple dors-lumbar vertebrae , left femur for follow up ,
- Osteoarthritic changes are seen at the both shoulders ,.
- Both kidneys are normally seen .

Conclusion:

Multiple active metastatic osseous lesions are seen as mentioned above for follow up

Nuclear Medicine Unit

DR . Sherien Mahdy . MCD

DR/ Mohamed Emam



Patient Name: Randa Ramzi Rezk Hanallah	Date : 25/02/2025
Exam : C.T CHEST WITH CONTRAST	Age : 61 Year(s)
Medical ID: 462/2025	Pat. ID: 97700

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN, AND PELVIS: -**

Findings:

I Chest:

- Left breast UIQ enhancing mass infiltrating the skin and measures about 6.8x3.5 cm.
- Multiple prominent left axillary lymph nodes seen at I, II levels, the largest at level I measures 1.4x1 cm.
- Tiny prominent left supraclavicular lymph node.
- Bilateral pulmonary fine atelectatic bands. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Heterogenous texture of the thyroid gland with bilobar hypodense nodules.

II Abdomen & pelvis:

- The liver is mildly enlarged in size cirrhotic appearance. No definite enhancing focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- The urinary bladder is seen distended showing diffuse mural thickening (5.6mm) with perivesical smudging of fat planes.
- Normal appearance of the other pelvic organs.



Patient Name: Randa Ramzi Rezk Hanallah	Date : 25/02/2025
Exam : C.T CHEST WITH CONTRAST	Age : 61 Year(s)
Medical ID: 462/2025	Pat. ID: 97700

➤ No abdominal or pelvic masses or collections.

***Scanned skeleton:**

- Bilateral ribs expanding destructive lytic lesions are noted with extra-osseous soft tissue components.
- L3 right pedicle lytic lesion with soft tissue comonenet measuers 2x1.8 cm has minimal intraspinal extenstion.
- Right iliac wing lytic lesion with outer cortical disruption.
- Sternal body and L4 vertebral body lytic lesions.
- Internal fixation of left femur by intramedullary nail and screws.

Opinion:

- Left breast locally advanced malignant mass with prominent left axillary lymph nodes.
- No pumonary or hepatic metastatic deposits.
- Multiple osseous metastatic deposits.
- Picture is suggestive of cysitis for urine analysis.

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Dr. Shaimaa Megahed, MSc

Dr. Mohamed Mohsen, M.D.

Patient's Name: Randa Ramzy Rizk Hana Allah
Date: 2026-08-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA.

I- CHEST:

- Left breast mass lesion is seen measuring 74 x 39 mm in maximum axial dimensions.
- Few bilateral scattered atelectatic bands.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing prominent caudate lobe. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple axial and appendicular skeleton lytic and sclerotic bone lesion.

Opinion:

- ❖ **Left breast lesion for sono-mammography.**
- ❖ **No pulmonary or hepatic metastatic lesions.**
- ❖ **Multiple axial and appendicular skeleton lytic and sclerotic bone lesions for bone scan.**

Dr.sherif Wagdy

Dr.sherif Wagdy

Patient's Name: Randa Ramzy Rizk Hana Allah
Date: 2026-08-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

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Dr.sherif Wagdy

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